



A CONVERSATION WITH DR. SANDRA SCOTT

Pull up a chair

The CEO of a Brooklyn safety net system on leading a hundred-year-old institution through the most transformative moment in modern healthcare — and why the seat you take determines the care you can deliver.



Dr. Sandra Scott

Chief Executive Officer
One Brooklyn Health



Eddie Qureshi

Founder & CEO, Rainfall Health
Host of the podcast

7,000+

employees across the
system

2 hospitals

serving central and east
Brooklyn

100 years

of history being asked to
change

She ran emergency medicine in one of the hardest-hit places in the country. Then she took the other seat.

Dr. Sandra Scott has spent her entire career in safety net institutions — Kings County, Lincoln in the South Bronx, and Brookdale, where she became the first woman, the first physician, and the first person of color to lead the hospital in its hundred-year history. She ran three emergency departments along the way, the last as chair of emergency medicine for One Brooklyn Health during COVID in New York City. In the spring of 2025 she was appointed CEO of the system.

That experience — being at the bedside while policy decided what the bedside could do — is what moved her. Being at the table where the decisions are made, she says, bringing her clinician experience and her cultural experience forward, was how she could actually advance care in central Brooklyn.

This guide distills her conversation with Eddie Qureshi on the Rainfall Health Podcast. It is written for clinicians weighing a leadership seat, for executives navigating a policy landscape nobody has a map for, and for anyone responsible for a safety net or rural institution being asked to transform faster than its funding allows.

“You have to be willing to pull up a chair and sit at the table, and not sit against the wall.”

DR. SANDRA SCOTT

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FEATURED VOICES

Dr. Sandra Scott

CEO of One Brooklyn Health, a safety net system serving central and east Brooklyn with more than 7,000 employees. An emergency physician by training and a national voice on Black women's maternal health.

Eddie Qureshi

Founder and CEO of Rainfall Health and host of the Rainfall Health Podcast.

COVID showed her where the decisions were actually made

She started as an emergency physician on the front lines, and every role since has been inside a safety net institution. She came up as a physician leader and ran three emergency departments; the last of those clinical leadership roles was chair of emergency medicine for One Brooklyn Health. She held it through COVID in New York City, in one of the hardest-hit communities in the country.

That experience opened her eyes to how policy and executive leadership shape the care delivered at the bedside. She says it plainly — and thank God she lived through it, because not everybody made it. What followed was a deliberate move toward the table: executive director of Brookdale Hospital, one of two hospitals in the system, and then, in the spring of 2025, CEO of the health system.

BEDSIDE

Thirty to a hundred patients

On an ER shift, the whole field of view is the individuals in front of you, one at a time.

DEPARTMENT

Staff, process, policy

As chair, the job becomes ensuring the care delivered at the bedside is high quality, efficient, and built from the patient's perspective.

SYSTEM

The community and the industry

Not only the communities you serve, but where the whole industry is moving — and how to be part of the change you want to see.

“It's kind of like driving the car, flying the plane — and then being in the spaceship, looking down at the world.”

DR. SANDRA SCOTT, ON THE CHANGE IN PERSPECTIVE

What never changes across those three altitudes, in her telling, is the thing at the center of her purpose: the patient. Everything else — scope, clock, vocabulary — is a matter of how far up you are sitting.

The air conditioning is a quality measure

Ask her what is different about running the hospital rather than the emergency department and she does not start with strategy. She starts with temperature and humidity — because operating rooms need a specific range depending on the procedure, and if the building can't hold it, the care can't happen. The same logic runs through environmental services: cleanliness is quality, so keeping EVS workers engaged is a clinical decision made in an operational chair.

WHAT STAYS THE SAME

Keeping the patient at the center of her purpose. That is the constant across every seat she has held, and it is the answer she gives before any other.

WHAT CHANGES

The scope, and what counts as within it. No longer hyper-focused on one specialty — now every system in the building is a variable in the quality of the care.

Where a clinician's eye keeps paying off

The building

Temperature, humidity, the physical plant. Infrastructure that determines which procedures the facility can safely run today.

The workforce

More than 7,000 employees across the system, including the people whose work never appears in a clinical note but shows up in every outcome.

The direction of travel

Where the industry is going nationally, and what part this system should play in getting the country to a healthier place.

It is a useful corrective for anyone building tools for health systems. The executive running a safety net hospital is not choosing between clinical priorities and operational ones. In her account they are the same list, and the list is much longer than the org chart suggests.

Brave enough to sit down, humble enough to learn

Eddie's standing line for clinicians is that if you're not at the table, you're on the menu. Her answer adds the harder part: getting there is not the achievement, and the risk does not disappear once you arrive. She lays out three conditions, and none of them are about credentials.

ONE

Be brave enough

Sit at the table rather than against the wall. The seat itself changes how engaged you can be in the conversation.

TWO

Be open-minded

Assume there is something you are going to learn. She is an emergency physician who says she learned something new about post-acute joint care in a single conversation.

THREE

Put it forward anyway

Take the risk that you might be wrong, or that no one understands your perspective. Diversity of perspective is what produces the better outcome.

“You also have to invite people to sit at the table. Literally. Sometimes people just need that invitation.”

DR. SANDRA SCOTT

It is the fourth condition she almost left out, and the only one aimed at people who already have a seat. When she sees someone take the back row, she asks them to move up. Eddie's read: she is not just taking the leadership mindset, she is handing it to everyone else in the room.

Nobody has run this experiment before

Asked to read the policy landscape from a safety net seat, she gives one word first: uncertainty. There are statistics that approximate what the reimbursement changes will do. Beyond that, she calls it largely uncharted territory — and uncertainty, as she puts it, is a breeding ground for fear and anxiety. There is a fair amount of both in the system right now.

Policy

Moving fast enough that reimbursement and models of care can only be forecast approximately. TEAM begins with 721 hospitals this year; by 2028 the expanded joint replacement model reaches every acute care hospital.

Technology

AI and automation arriving without settled answers on workforce impact, or on whether patients will access care in a way that has no precedent in centuries of practice.

Policy on technology

A third layer she is watching in the media: growing governmental influence over which technological advancements health systems will have access to at all.

“What we do know is change is upon us. This will likely be some of the most transformative change in healthcare in modern history.”

DR. SANDRA SCOTT

What she owes her organization in the middle of that

A message, and the willingness to stand behind it: we are changing, and we have no option not to change. Uncertainty about the destination is not a reason to stall — it is the reason the message has to be unambiguous.

And a definition of what transformation actually means. Not new language for the same operation, but an admission that the way care has been delivered historically cannot be the way it is delivered next.

Grandpa, you can't smoke your pipe by the fireplace anymore

That is her analogy for delivering a transformation message to a hundred-year-old institution, and she offers it with the timing of someone who has tried. Imagine having that conversation with a relative who has done something the same way their whole life. How do you think it goes?

Patients want to access healthcare in a myriad of different ways. If we can't diversify how we deliver care to meet the needs of the communities we serve, we're going to be left behind — and when you're left behind, eventually you're not open.

DR. SANDRA SCOTT

Four front doors, all of them legitimate

Telehealth

For patients who want clinical contact without the trip.

An AI agent

Some people genuinely prefer it to speaking with a human being. She says so without hedging.

Brick and mortar

Still the choice of many, and not going away.

The home

Services that come to the patient rather than the reverse.

Eddie's takeaway

Meet the patient where they are. Don't force them toward what we think they should do. The institution is the entity that should change — and her emergency background shows up in every answer, because the patient keeps landing at the center of it.

The obligation underneath

A safety net system has to run a fiscally sound business in order to serve its community at all. That is the hard part of the message, and she does not soften it for the institution's age.

Safety net and rural hospitals are in the same squeeze

She will not answer this one about safety net systems alone. Rural hospitals belong in the same sentence, she says, and too often they are left out of the conversation entirely. The mechanics are identical: subsidies and financial support from government entities, a rising cost base in supplies and labor, and reimbursement models that have not kept pace with either.

THE ARITHMETIC

There's less money to go around, and there are more of us who need it.

More institutions across the country now need additional support simply to stay whole, pay their bills, and keep the lights on.

THE RESPONSE

Stop duplicating what the hospital a few miles away already does well.

Two formerly competing institutions both running the same service line is a total-cost-of-care problem the community pays for twice.

What is actually on the table

Collaborate

Provide a shared service jointly, so the cost of delivering it to the community falls rather than doubles.

Close and coordinate

Close a service another institution already provides, send patients there deliberately, and take them back for follow-up care.

Transform outright

Change how care is delivered and who it is delivered with, because the gaps cannot be filled at the current cost structure.

“The pressure to transform, the pressure to change, is intense and imminent and upon us now. We don't have the luxury of taking ten years.”

DR. SANDRA SCOTT

Her colleagues, she says, are feeling exactly the same pressure — the changes have to be made incrementally and now, in order to keep delivering quality care to the communities these institutions exist to serve.

The policy makes sense. The implementation was left as an exercise.

Asked what she would tell the people writing care delivery policy, she starts by conceding the concept. Reading the TEAM model, care coordination makes sense — conceptually, and to her as a clinician. Her objection is not to the goal. It is that the coordination required to actually reap the outcomes the policy wants has no infrastructure behind it.

We are not used to working together

That is the honest starting condition, in her words. Healthcare is not practiced at pulling together as a team. COVID changed a great deal of it — there was no choice, and systems learned to load-balance patients toward whoever had capacity — but the default did not permanently reset.

Her analogy is a workshop. Bring a room full of healthcare leaders together to solve a problem and it still needs to be facilitated. Everyone in it is smart and capable; aligning their mindsets around one common goal is a separate skill, and someone has to supply it.

“I feel a little bit like that's the missing link — health systems are somewhat on their own to figure it out.”

DR. SANDRA SCOTT

Her ask of policymakers is therefore narrow and practical: think about how to facilitate implementation, not only how to define the target. The gap between a sound model and a realized outcome is not clinical judgment. It is coordination nobody has been funded or equipped to run.

What the policy supplies

A goal everyone can agree with, a defined episode, and a set of outcomes that will be measured and paid against.

What it does not

Any mechanism for facilitating the collaboration the goal depends on — across institutions that have no history of coordinating and no shared rails to do it on.

What that produces

Misalignment across exactly the institutions that could have worked together to deliver the outcome the policy was written to produce.

The chair metaphor, applied to community organizations

She is an open fan of community-based organizations and the services they provide in support of patient care. She is equally clear about how they get treated. Historically those small organizations are not considered — because they're small, because they're in the community, and because, as she puts it, we're the big old health system. And they have no easy way to engage with us even when they want to.

THE ANCHOR

One institution sees most of the community

In most communities an acute care hospital touches well over 70% of the population at some point. One Brooklyn Health is that anchor for central and east Brooklyn.

THE CONTINUUM

Most of the care happens somewhere else

Under a bundled model, more than 60% of the care in an episode occurs outside the acute setting — in therapy clinics, primary care, and community organizations the hospital does not run.

“What if there was a more sophisticated way, perhaps through technology, that automatically brings us together around a common goal?”

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Why the invitation has to be structural

Goodwill does not scale. A CEO can invite a community organization into one conversation; an episode of care requires dozens of those handoffs to happen reliably, for every patient, without anyone making a phone call to arrange it.

Which makes this the same idea as chapter three, moved from the boardroom to the infrastructure. The organizations sitting against the wall have something the episode needs. Someone has to pull up their chair.

Interoperability has been a buzzword for a decade

Eddie pushed the point one click deeper. The anchor hospital and the community organizations have to work collectively to deliver the episode — and the infrastructure to do it still doesn't exist. Interoperability has been promised for years and has never come close to fruition. Her agreement was immediate and unqualified.

“It can't be faxes and emails and phone calls. That doesn't work. If it ever worked.”

DR. SANDRA SCOTT

What has to evolve, in order

THE POLICY

Already moving, and conceptually sound. The models name the goal and the accountable party.

THE PROCESS

Health systems can and are redesigning how they discharge, refer, and follow up. Necessary, and not sufficient.

THE INFRASTRUCTURE

The piece nobody has built: rails that connect an anchor hospital to every other party in the episode, automatically.

It is worth noticing what this reframes. The bundled payment conversation usually gets held as a policy problem or a behavior problem. Her read is that it is a plumbing problem wearing a policy costume — and the institutions being held accountable for ninety days of care are the ones least equipped to lay the pipe themselves.

Where a safety net CEO is finding hope

Asked the closing question, she doesn't hesitate: the technological advancements. She used to dream about robots handling the mundane, repetitive work — why is a physician carrying a tube of blood across the emergency department, and why does a human being have to move linen to the floors and back every single day? It is already happening in some hospitals, freeing people for the human touch a robot cannot yet provide.

THE EQUITY POINT

Technology is coming to safety net systems too — this is not a haves and have-nots moment.

She is implementing AI and exploring robotics at One Brooklyn Health now, and describes the point in time as one where everyone will have access.

The ten minutes, and the handoff that follows

Her example of the frustration is specific to her specialty. A patient arrives from the community. Nine times out of ten — she corrects herself, nine point eight — she has never met them, and she has roughly ten minutes to understand them. Then, at the end of the encounter, she should be able to send everything relevant back out to the people in the community who will help that person heal. With one click.

Instead: a stack of referrals, phone calls, and extra typing into the computer.

Her version of the future is almost mundane: an ankle sprain, one follow-up button. The appointment is made, primary care knows the medication, the pharmacy is ready — because the record knows who the patient is. And an AI agent calls when they get home, just to make sure they understand the care plan.

Why it counts as hope

It is the interoperability conversation arriving at the bedside, in a form a clinician can actually feel. She calls it plausible, and says she believes we're reaching the point where it will be reality.

Eddie's closing note

That future isn't a decade out. Much of it is here right now — which puts the burden back on the institutions to be ready to use it.

The facilitation layer she says is missing

Rainfall Health is an accountability and accessibility platform powered by AI — the first and only company certified for these Medicare-mandated programs. The gap Dr. Scott describes is precisely the one it was built to close: policy that makes sense, institutions willing to coordinate, and nothing underneath them that makes coordination automatic.

Everyone at the table

Community-based organizations, post-acute partners, and primary care connected into the same episode as the anchor hospital — without depending on faxes, emails, or a phone call somebody has to remember to make.

Support that reaches the home

Around-the-clock patient and caregiver support with escalation pathways — the advocacy, education, and follow-through that determine whether a discharge holds.

Built for the mandate

Patient-reported outcomes, quality-gate metrics, and risk-adjusted episode performance across the full 30- or 90-day window — as one system rather than a stack of vendors.

FROM THE CONVERSATION

“What if there was a more sophisticated way, perhaps through technology, that automatically brings us together around a common goal?”

DR. SANDRA SCOTT — THE DESIGN BRIEF, STATED AS A QUESTION

Her second condition matters as much as the first. A safety net system in central Brooklyn and a rural hospital with one orthopedic group cannot be served by different products — the variability she describes is the design constraint, and building for both at once is the only version worth building.

GET CONNECTED

Pull up a chair with us

If you're leading a safety net, rural, or community health system through this transition — connecting community-based organizations to the episode, capturing patient-reported outcomes, or working out how coordination holds together across parties who don't share a chart — we'd like to help. Rainfall is the end-to-end solution, technology and services, for hospitals and health systems.

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Watch for the next episode of the Rainfall Health Podcast.



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